

numb the area. If a persistent missed segment is too uncomfortable, the anesthesiologist may do a combined spinal epidural block, known as a [CSE](#).

It's thought that epidural pain relief may prolong the second stage of labor. It also increases your chances of having an assisted delivery, especially if a high dose of anesthetic is given toward the end of labor, which affects your ability to push. However, an epidural doesn't increase the chance of a cesarean and doesn't, despite common misconceptions, cause long-term backache after the birth.

THE PROS AND CONS OF AN EPIDURAL

Pros

- An epidural provides absolute pain relief in 90 percent of cases; 10 percent of women have some degree of residual pain but still have a marked improvement in their overall discomfort.
- Epidurals do not pose a risk to the baby.
- The presence of an effective epidural means that if intervention is needed at any time, the epidural can be topped up with an anesthetic for either an [assisted delivery](#), or a [Cesarean delivery](#). This also reduces the likelihood that a general anesthetic will be needed.

Cons

- Around 1 in 10 women do not experience absolute pain relief with an epidural.
- Some women develop a headache that persists after an epidural (see [An epidural "headache"](#)).
- A rare complication is patches of heaviness in the legs or feet.

There are some very rare risks:

- As with all invasive procedures, inserting an epidural can result in infection. Meningitis occurs in around 1 in 100,000 women and an epidural abscess occurs in about 1 in 50,000 women.
- There is a 1 in 170,000 risk of developing a blood clot in the epidural space (epidural hematoma).
- There is a 1 in 100,000 risk of the epidural tube moving into the fluid around the spine and resulting in unconsciousness, and there is a 1 in 250,000 chance of the epidural causing some form of paralysis.

HOW AN EPIDURAL IS DONE

If you opt for an epidural, the doctor should explain the procedure to you, and you should have the opportunity to ask the anesthesiologist any questions.

Getting ready for an epidural Before starting the epidural, a plastic tube will be placed in a vein in the back of your hand or in your arm, to which an IV containing fluid will be connected. You are given fluids during an epidural to stop your blood pressure from dropping. The doctor will then help you into the correct position to receive the epidural, which will either be sitting up with your legs over the side of the bed leaning forward, or curled up on your side on the edge of the bed. The position may depend on the preference of the anesthesiologist.

Your lower back will be cleaned with antiseptic and a drape placed over the rest of your back to reduce the risk of infection. Before the epidural needle is inserted, a local anesthetic will be given into the skin and surrounding tissues. This creates a numb patch to ensure that the insertion of the large epidural needle is not painful. When the local anesthetic is injected, you may feel a scratching sensation and experience a very short-lived sting in the area between the vertebrae bones.